There is a deep conceptual problem. The neutral application loophole does not misapply the sameness-difference conception, as sympathetic courts and advocates have attempted to show, but constitutes its *reductio ad absurdum*.

My diagnosis

same cannot be said here. The Plan does not merely exclude one "objectively identifiable physical condition with unique characteristics" from coverage; rather, it excludes *treatments* that lead or are connected to *sex* changes or modifications. Pregnancy can be explained without reference to sex, gender, or transgender status.[7] The same cannot be said of the exclusion at issue here.

The Court is not persuaded by Defendant's arguments. First, inherent in a gender dysphoria diagnosis is a person's identity as transgender. In other words, a person cannot suffer from gender dysphoria without identifying as transgender. *See Kadel*, 2022 WL 2106270, at *20 ("even if the Court credited

What does that mean in practice? Simply that sex determines access to the covered medication. Physicians in Tennessee can prescribe hormones and puberty blockers to help a male child, but not a female child, look more like a boy; and to help a female child, but not a male child, look more like a girl. Put in the statute's own terms, doctors can facili-

After *Price Waterhouse,* an employer who discriminates against women because, for instance, they do not wear dresses or makeup, is engaging in sex discrimination because the discrimination would not occur but for the victim's sex. It follows that employers who discriminate against men because they *do* wear dresses and makeup, or otherwise act femininely, are also engaging in sex discrimination, because the discrimination would not occur but for the victim's sex. *See, e.g., Nichols,* 256 F.3d

into resigning. In short, Smith claims that the discrimination he experienced was based on his failure to conform to sex stereotypes by expressing less masculine, and more feminine mannerisms and appearance.

same cannot be said here. The Plan does not merely exclude "objectively identifiable physical condition with unique charac istics" from coverage; rather, it excludes *treatments* that lea are connected to *sex* changes or modifications. Pregnancy ca explained without reference to sex, gender, or transgender tus.[7] The same cannot be said of the exclusion at issue here.

There is a deep conceptual problem. The neut

The Court is not persuaded by Defendant's arguments. First, in gender dysphoria diagnosis is a person's identity as transgende words, a person cannot suffer from gender dysphoria without ide transgender. *See Kadel*, 2022 WL 2106270, at *20 ("even if the Cou

What does that mean in practice? Simply that sex determines access to the covered medication. Physicians in Tennessee can prescribe hormones and puberty blockers to help a male child, but not a female child, look more like a boy; and to help a female child, but not a male child, look more like a girl. Put in the statute's own terms, doctors can facili-

into resigning. In short, Smith claims that the discrimination he experienced was based on his failure to conform to sex stereotypes by expressing less masculine, and more feminine mannerisms and appearance.

After *Price Waterhouse*, an employer who discriminates against women because, for instance, they do not wear dresses or makeup, is engaging in sex discrimination because the discrimination would not occur but for the victim's sex. It follows that employers who discriminate against men because they *do* wear dresses and makeup, or otherwise act femininely, are also engaging in sex discrimination, because the discrimination would not occur but for the victim's sex. *See, e.g., Nichols*, 256 F.3d

We are still working under a substantively cis-centric—even if ostensibly trans-inclusive!—framework of gender equality under which trans women are analyzed in one way or another as gender-nonconforming males.